Referral packet F10: readable copy

Source: F10.pdf (fictional), 14 pages. Left side of each sheet is the original page, right side is a typed transcript. Black boxes are privacy redactions, not missing information.

Page map

Page	Document	Type	Legibility	Check by hand
1	Fax cover sheet	fax_cover	good	
2	IHH Recuperative Care Referral Form	referral_form	good	
3	CalAIM Community Supports authorization request	calaim_auth	good	
4	Patient face sheet	face_sheet	good	
5	History and Physical - page 1	h_and_p	good	
6	History and Physical - page 2	h_and_p	good	
7	Wound care nurse note	wound_note	good	
8	Hospitalist progress note	progress_note	good	
9	Case management and social work note	social_work	good	
10	Physical therapy evaluation	pt_eval	good	
11	Occupational therapy evaluation	ot_eval	good	
12	Glucose log and vital signs	vitals	good	
13	Laboratory results	labs	good	
14	Discharge medication list	med_list	good	

Fax cover sheet

Lakeshore General Hospital (fictional)
Case Management Department
FAX TRANSMITTAL
To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Carla Mendes, RN CM
Phone: 555-0150
Date: 06/12/2026 08:45
Pages (including cover): 14
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]
Message
58M T2DM admitted for hyperglycemia, now controlled on oral agents.
Ready for d/c 06/12 PM. PT/OT, wound, SW notes and glucose log attached.
Please call CM with questions.
CONFIDENTIALITY NOTICE
This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 1: Fax cover sheet

fax_cover | orientation normal | legibility good

Lakeshore General Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Carla Mendes, RN CM
Phone: 555-0150
Date: 06/12/2026 08:45
Pages (including cover): 14
Re: Recuperative care referral
Urgent **[X]** For Review **[X]** Please Reply **[X]**

Message

58M T2DM admitted for hyperglycemia, now controlled on oral agents.
Ready for d/c 06/12 PM. PT/OT, wound, SW notes and glucose log attached.
Please call CM with questions.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

IHH Recuperative Care Referral Form

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Lakeshore General Hospital (fictional)
Department/Floor: 6 East Medical
Social Worker/Case Manager/Nurse: Carla Mendes, RN CM
Phone: 555-0150
Email: cmendes@lakeshoregen.example
Nursing Station: 555-0151
Authorized By: Dr. N. Achterberg

Patient Information

Name: Daniel Doe
MRN: LG-0572214
Birth Date: 02/09/1968
Gender: M
Ethnicity: White
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English
Housing Situation:
Currently Unhoused [X]
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Hyperglycemia, uncontrolled T2DM
Admit Date: 06/08/2026
Any Wounds: Yes [ ] No [X]
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes [X] No [ ]
PPD/TB Test or Chest X-Ray: Date: 06/08/2026 Result: CXR no acute process

History of Health & Living Situation

O2: Yes [ ] No [X]
Self Administer All Meds: Yes [X] No [ ]
Continent of Bowel & Bladder: Yes [X] No [ ]
Colostomy/Ileostomy: Yes [ ] No [X]
Foley Catheter: Yes [ ] No [X]
Diabetes: Yes [X] No [ ]
Insulin: Yes [ ] No [X]
Diabetes Oral Meds: Yes [X] No [ ]
Active Infection: Yes [ ] No [X]
Ambulatory: Yes [X] No [ ]
Assistive Device: Yes [X] No [ ] cane
Hospice: Yes [ ] No [X]
Car: Yes [ ] No [X]
Service Animal: Yes [ ] No [X]

Behavioral and Mental Health Status

Any limitations or challenges: No [X] Yes [ ]

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 2: IHH Recuperative Care Referral Form

referral_form | orientation normal | legibility good

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Lakeshore General Hospital (fictional)
Department/Floor: 6 East Medical
Social Worker/Case Manager/Nurse: Carla Mendes, RN CM
Phone: 555-0150
Email: cmendes@lakeshoregen.example
Nursing Station: 555-0151
Authorized By: Dr. N. Achterberg

Patient Information

Name: Daniel Doe
MRN: LG-0572214
Birth Date: 02/09/1968
Gender: M
Ethnicity: White
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English
Housing Situation:
Currently Unhoused **[X]**
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Hyperglycemia, uncontrolled T2DM
Admit Date: 06/08/2026
Any Wounds: Yes [ ] No **[X]**
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes **[X]** No [ ]
PPD/TB Test or Chest X-Ray: Date: 06/08/2026 Result: CXR no acute process

History of Health & Living Situation

O2: Yes [ ] No **[X]**
Self Administer All Meds: Yes **[X]** No [ ]
Continent of Bowel & Bladder: Yes **[X]** No [ ]
Colostomy/Ileostomy: Yes [ ] No **[X]**
Foley Catheter: Yes [ ] No **[X]**
Diabetes: Yes **[X]** No [ ]
Insulin: Yes [ ] No **[X]**
Diabetes Oral Meds: Yes **[X]** No [ ]
Active Infection: Yes [ ] No **[X]**
Ambulatory: Yes **[X]** No [ ]
Assistive Device: Yes **[X]** No [ ] cane
Hospice: Yes [ ] No **[X]**
Car: Yes [ ] No **[X]**
Service Animal: Yes [ ] No **[X]**

Behavioral and Mental Health Status

Any limitations or challenges: No **[X]** Yes [ ]
Mental Health DX: Yes [ ] No **[X]** Type:
Non-compliant: Yes [ ] No **[X]**
Cognitive Impairment: Yes [ ] No **[X]**
Forgetful: Yes [ ] No **[X]**
Registered Sex Offender: Yes [ ] No **[X]**
Other: none

Length of Stay Authorized

Days: 14

Illumination Staff Only

Date Received:
Reviewed By:
Approved: Yes [ ] No [ ]
Date:
Authorization #:
Reason for Denial:
Admission Date/Time:
Exit Date:
Extension Request: Yes [ ] No [ ]

-- FICTIONAL DEMO RECORD - not a real patient --

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST
Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal
Member
Member Name: Daniel Doe
DOB: 02/09/1968
Member ID: OCH-61842097
Referring Provider
Facility: Lakeshore General Hospital (fictional)
Clinical
Primary Diagnosis: Type 2 diabetes mellitus with hyperglycemia
ICD-10: E11.65
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 06/12/2026
Requested Days: 14
Secondary Dx: Type 2 DM with foot ulcer (E11.621); non-pressure chronic ulcer R foot, limited to breakdown of skin (L97.511)
Plan Use Only
Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 3: CalAIM Community Supports authorization request

calaim_auth | orientation normal | legibility good

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Daniel Doe
DOB: 02/09/1968
Member ID: OCH-61842097

Referring Provider

Facility: Lakeshore General Hospital (fictional)

Clinical

Primary Diagnosis: Type 2 diabetes mellitus with hyperglycemia
ICD-10: E11.65
Member is experiencing homelessness **[X]**
Member at risk of hospitalization/re-hospitalization **[X]**
Requested Start Date: 06/12/2026
Requested Days: 14
Secondary Dx: Type 2 DM with foot ulcer (E11.621); non-pressure chronic ulcer R foot, limited to breakdown of skin (L97.511)

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Patient face sheet

Lakeshore General Hospital (fictional) - PATIENT FACE SHEET
Patient: Daniel Doe
MRN: LG-0572214
Account #: 99572214
DOB: 02/09/1968
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)
Encounter
Admit Date/Time: 06/08/2026 19:05 (ED arrival 06/08/2026 14:20)
Admit Type: Inpatient
Unit/Bed: 6E-621A
Attending: Nora Achterberg, MD
Insurance
Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self
Emergency Contact
None listed

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 4: Patient face sheet

face_sheet | orientation normal | legibility good

Lakeshore General Hospital (fictional) - PATIENT FACE SHEET

Patient: Daniel Doe
MRN: LG-0572214
Account #: 99572214
DOB: 02/09/1968
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 06/08/2026 19:05 (ED arrival 06/08/2026 14:20)
Admit Type: Inpatient
Unit/Bed: 6E-621A
Attending: Nora Achterberg, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 1

HISTORY AND PHYSICAL
Date of Service: 06/08/2026
Attending: Nora Achterberg, MD (Hospital Medicine)
Chief Complaint
"My sugar has been over 400 on the shelter meter" x 1 week, polyuria, thirst
HPI
58 y/o M with T2DM (dx 2019) experiencing homelessness (sleeps at Example Shelter, Santa Ana, x 8 months) presents with 1 week of polyuria, polydipsia and fatigue after running out of metformin 3 weeks ago. Also reports a sore on the bottom of his right foot x 3 weeks, painless, no drainage, no fever. In ED: POC glucose 438, anion gap 11, serum beta-hydroxybutyrate 0.3 (negative). Not in DKA. No HHS (serum osm 296).
PMH
- T2DM, last Alc unknown prior to this admission
- Peripheral neuropathy, bilateral feet
- Hyperlipidemia
PSH
- None
Social History
Homeless x 8 months, staying at shelter. Former smoker (quit 2020). Denies alcohol or drug use.
Allergies
Sulfa (hives)
Home Medications
- Metformin 1000 mg PO BID (not taking x 3 weeks)
- Atorvastatin 20 mg PO daily

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 5: History and Physical - page 1

h_and_p | orientation normal | legibility good

HISTORY AND PHYSICAL

Date of Service: 06/08/2026
Attending: Nora Achterberg, MD (Hospital Medicine)

Chief Complaint

"My sugar has been over 400 on the shelter meter" x 1 week, polyuria, thirst

HPI

58 y/o M with T2DM (dx 2019) experiencing homelessness (sleeps at Example Shelter, Santa Ana, x 8 months) presents with 1 week of polyuria, polydipsia and fatigue after running out of metformin 3 weeks ago. Also reports a sore on the bottom of his right foot x 3 weeks, painless, no drainage, no fever. In ED: POC glucose 438, anion gap 11, serum beta-hydroxybutyrate 0.3 (negative). Not in DKA. No HHS (serum osm 296).

PMH

- T2DM, last A1c unknown prior to this admission
- Peripheral neuropathy, bilateral feet
- Hyperlipidemia

PSH

- None

Social History

Homeless x 8 months, staying at shelter. Former smoker (quit 2020). Denies alcohol or drug use.

Allergies

Sulfa (hives)

Home Medications

- Metformin 1000 mg PO BID (not taking x 3 weeks)
- Atorvastatin 20 mg PO daily

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 2

Physical Exam
VS: T 98.9F, HR 96, BP 132/80, RR 16, SpO2 98% RA
Gen: NAD, AOx4
CV: RRR, no m/r/g
Lungs: CTAB
Skin/Feet: R plantar 1st metatarsal head ulcer approx 1.6 x 1.3 cm, shallow, clean base, no erythema, no warmth, no drainage, no odor. Probe-to-bone negative. Decreased monofilament sensation bilat. DP pulses 2+ bilat.
Psych: calm, cooperative, linear thought process
Assessment / Plan
58M with uncontrolled T2DM with hyperglycemia without DKA, and non-infected neuropathic R plantar foot ulcer.
1. Hyperglycemia - IVF, resume metformin, add sitagliptin; POC glucose AC/HS. No insulin planned unless persistent >300.
2. R foot ulcer, non-infected - X-ray R foot to r/o osteomyelitis; wound care consult; offloading post-op shoe; no antibiotics indicated
3. MRSA nares PCR per protocol
4. Dispo - CM consult; homeless, anticipate recuperative care referral
DVT ppx: enoxaparin 40 mg SC daily
Isolation: None - standard precautions
Code status: Full
Electronically signed: Nora Achterberg, MD 06/08/2026 21:30

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 6: History and Physical - page 2

h_and_p | orientation normal | legibility good

Physical Exam

VS: T 98.9F, HR 96, BP 132/80, RR 16, SpO2 98% RA
Gen: NAD, AOx4
CV: RRR, no m/r/g
Lungs: CTAB
Skin/Feet: R plantar 1st metatarsal head ulcer approx 1.6 x 1.3 cm, shallow, clean base, no erythema, no warmth, no drainage, no odor. Probe-to-bone negative. Decreased monofilament sensation bilat. DP pulses 2+ bilat.
Psych: calm, cooperative, linear thought process

Assessment / Plan

58M with uncontrolled T2DM with hyperglycemia without DKA, and non-infected neuropathic R plantar foot ulcer.
1. Hyperglycemia - IVF, resume metformin, add sitagliptin; POC glucose AC/HS. No insulin planned unless persistent >300.
2. R foot ulcer, non-infected - X-ray R foot to r/o osteomyelitis; wound care consult; offloading post-op shoe; no antibiotics indicated
3. MRSA nares PCR per protocol
4. Dispo - CM consult; homeless, anticipate recuperative care referral
DVT ppx: enoxaparin 40 mg SC daily
Isolation: None - standard precautions
Code status: Full
Electronically signed: Nora Achterberg, MD 06/08/2026 21:30

-- FICTIONAL DEMO RECORD - not a real patient --

Wound care nurse note

WOUND CARE CONSULT NOTE
Date: 06/11/2026 10:20
Wound #1: R plantar foot, 1st metatarsal head, diabetic neuropathic ulcer, Wagner grade 1
Size: 1.4 cm L x 1.1 cm W x 0.2 cm D (improved from 1.6 x 1.3 on 06/09)
Wound bed: 100% pink granulation. Drainage: scant serous. Odor: none
Periwound: callus trimmed, no erythema, no induration, no signs of infection
Treatment
- Cleanse with NS, apply silver alginate, cover with bordered foam dressing
- Change dressing every other day and PRN; last changed 06/11/2026 AM
- Offloading: post-op shoe R foot at all times when walking
Education
Pt return-demonstrated dressing change independently x2 and daily foot self-inspection with mirror.
Recommendation
Home health RN wound check 3x/week (to match every-other-day dressing changes). Podiatry follow-up in 1-2 weeks.
Signed: G. Ilunga, RN CWOCN

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 7: Wound care nurse note

wound_note | orientation normal | legibility good

WOUND CARE CONSULT NOTE

Date: 06/11/2026 10:20
Wound #1: R plantar foot, 1st metatarsal head, diabetic neuropathic ulcer, Wagner grade 1
Size: 1.4 cm L x 1.1 cm W x 0.2 cm D (improved from 1.6 x 1.3 on 06/09)
Wound bed: 100% pink granulation. Drainage: scant serous. Odor: none
Periwound: callus trimmed, no erythema, no induration, no signs of infection

Treatment

- Cleanse with NS, apply silver alginate, cover with bordered foam dressing
- Change dressing every other day and PRN; last changed 06/11/2026 AM
- Offloading: post-op shoe R foot at all times when walking

Education

Pt return-demonstrated dressing change independently x2 and daily foot self-inspection with mirror.

Recommendation

Home health RN wound check 3x/week (to match every-other-day dressing changes). Podiatry follow-up in 1-2 weeks.
Signed: G. Ilunga, RN CWOCN

-- FICTIONAL DEMO RECORD - not a real patient --

Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5
Date: 06/12/2026 07:30
S: Feels well, thirst resolved. Walking halls with cane. No foot pain.
O: T 98.2F HR 78 BP 124/76 RR 16 SpO2 98% RA
Glucose 118-152 over last 48h on metformin + sitagliptin. No hypoglycemia this admission. No insulin given this admission.
R foot X-ray 06/09: no osseous erosion, no osteomyelitis, no gas.
R foot ulcer improving per wound RN, no signs of infection.
A1c 9.6% (06/09).
A/P:
- T2DM with hyperglycemia, now controlled on oral agents. Continue metformin 1000 mg BID + sitagliptin 100 mg daily
- R plantar diabetic foot ulcer, non-infected, healing - dressing every other day, home health wound RN, podiatry f/u
- Medically stable for d/c to recuperative care today
Signed: Nora Achterberg, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 8: Hospitalist progress note

progress_note | orientation normal | legibility good

HOSPITALIST PROGRESS NOTE - HD#5

Date: 06/12/2026 07:30
S: Feels well, thirst resolved. Walking halls with cane. No foot pain.
O: T 98.2F HR 78 BP 124/76 RR 16 SpO2 98% RA
Glucose 118-152 over last 48h on metformin + sitagliptin. No hypoglycemia this admission. No insulin given this admission.
R foot X-ray 06/09: no osseous erosion, no osteomyelitis, no gas.
R foot ulcer improving per wound RN, no signs of infection.
A1c 9.6% (06/09).
A/P:

- T2DM with hyperglycemia, now controlled on oral agents. Continue metformin 1000 mg BID + sitagliptin 100 mg daily
- R plantar diabetic foot ulcer, non-infected, healing - dressing every other day, home health wound RN, podiatry f/u
- Medically stable for d/c to recuperative care today

Signed: Nora Achterberg, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Case management and social work note

CASE MANAGEMENT / SOCIAL WORK NOTE
Date: 06/12/2026 09:10
Pt AOx4, pleasant and cooperative throughout stay, no behavioral concerns per nursing.
Psychosocial screen: denies depression. Denies suicidal ideation. Denies homicidal ideation. No hallucinations reported or observed. No psychiatric history. No history of conservatorship or FSP.
Substance use: denies alcohol and drug use; no withdrawal signs.
Pt agreeable to recuperative care.
Arrangements
Home health referral accepted by Bayside Home Health (fictional), 555-0156; RN start of care visit scheduled 06/13/2026 for wound check and dressing change, then 3x/week.
Podiatry appointment: Lakeshore Podiatry Clinic (fictional) 06/22/2026 at 10:00 (confirmed).
PCP follow-up: Lakeshore Community Clinic (fictional) 06/18/2026 at 14:30 (confirmed) for diabetes.
Signed: Carla Mendes, RN CM

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 9: Case management and social work note

social_work | orientation normal | legibility good

CASE MANAGEMENT / SOCIAL WORK NOTE

Date: 06/12/2026 09:10
Pt AOx4, pleasant and cooperative throughout stay, no behavioral concerns per nursing.
Psychosocial screen: denies depression. Denies suicidal ideation. Denies homicidal ideation. No hallucinations reported or observed. No psychiatric history. No history of conservatorship or FSP.
Substance use: denies alcohol and drug use; no withdrawal signs.
Pt agreeable to recuperative care.

Arrangements

Home health referral accepted by Bayside Home Health (fictional), 555-0156; RN start of care visit scheduled 06/13/2026 for wound check and dressing change, then 3x/week.
Podiatry appointment: Lakeshore Podiatry Clinic (fictional) 06/22/2026 at 10:00 (confirmed).
PCP follow-up: Lakeshore Community Clinic (fictional) 06/18/2026 at 14:30 (confirmed) for diabetes.
Signed: Carla Mendes, RN CM

-- FICTIONAL DEMO RECORD - not a real patient --

Physical therapy evaluation

PHYSICAL THERAPY EVALUATION
Date: 06/10/2026 13:00
Dx: R plantar foot ulcer, peripheral neuropathy
Prior Level of Function
Independent ambulation, used a single-point cane outdoors for the past year.
Assessment
Bed mobility: Independent
Sit<>stand: Independent
Transfers: Independent
Gait: 150 ft with single-point cane in R post-op shoe, modified independent, no rest breaks, steady
Stairs: 6 steps with rail, modified independent
Balance: static good, dynamic good-
Plan / Recommendation
Safe for d/c to recuperative care with own cane and post-op shoe. Uses cane independently. No PT follow-up needs.
Signed: H. Varga, PT, DPT

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 10: Physical therapy evaluation

pt_eval | orientation normal | legibility good

PHYSICAL THERAPY EVALUATION

Date: 06/10/2026 13:00
Dx: R plantar foot ulcer, peripheral neuropathy

Prior Level of Function

Independent ambulation, used a single-point cane outdoors for the past year.

Assessment

Bed mobility: Independent
Sit<>stand: Independent
Transfers: Independent
Gait: 150 ft with single-point cane in R post-op shoe, modified independent, no rest breaks, steady
Stairs: 6 steps with rail, modified independent
Balance: static good, dynamic good-

Plan / Recommendation

Safe for d/c to recuperative care with own cane and post-op shoe. Uses cane independently. No PT follow-up needs.
Signed: H. Varga, PT, DPT

-- FICTIONAL DEMO RECORD - not a real patient --

Occupational therapy evaluation

OCCUPATIONAL THERAPY EVALUATION
Date: 06/10/2026 15:15
ADLs
Grooming: Independent
UB dressing: Independent
LB dressing: Independent, dons post-op shoe independently
Toileting: Independent
Shower: Independent seated on shower bench, keeps R foot dressing dry with cast cover; gathers own supplies
Cognition: AOx4, follows multi-step commands, manages own meds with pill organizer
Recommendation
D/c to recuperative care. No further OT needs.
Signed: P. Okonkwo, OTR/L

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 11: Occupational therapy evaluation

ot_eval | orientation normal | legibility good

OCCUPATIONAL THERAPY EVALUATION

Date: 06/10/2026 15:15

ADLs

Grooming: Independent
UB dressing: Independent
LB dressing: Independent, dons post-op shoe independently
Toileting: Independent
Shower: Independent seated on shower bench, keeps R foot dressing dry with cast cover; gathers own supplies
Cognition: AOx4, follows multi-step commands, manages own meds with pill organizer

Recommendation

D/c to recuperative care. No further OT needs.
Signed: P. Okonkwo, OTR/L

-- FICTIONAL DEMO RECORD - not a real patient --

Glucose log and vital signs

POINT-OF-CARE GLUCOSE - LAST 5 CONSECUTIVE

Date/Time | Glucose (mg/dL) | Insulin given
06/10 21:00 | 214 | none
06/11 07:00 | 176 | none
06/11 11:30 | 152 | none
06/11 21:00 | 141 | none
06/12 07:00 | 118 | none

No hypoglycemia this admission.
Pt checks own glucose with glucometer independently (teach-back 06/10, RN T. Lam).

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
06/11 08:00 | 98.4 | 82 | 128/78 | 16 | 98% | RA
06/11 12:00 | 98.6 | 80 | 126/78 | 16 | 97% | RA
06/11 20:00 | 98.3 | 76 | 122/74 | 16 | 98% | RA
06/12 00:00 | 98.0 | 74 | 120/72 | 14 | 98% | RA
06/12 07:30 | 98.2 | 78 | 124/76 | 16 | 98% | RA

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 12: Glucose log and vital signs

vitals | orientation normal | legibility good

POINT-OF-CARE GLUCOSE - LAST 5 CONSECUTIVE

Date/Time | Glucose (mg/dL) | Insulin given
06/10 21:00 | 214 | none
06/11 07:00 | 176 | none
06/11 11:30 | 152 | none
06/11 21:00 | 141 | none
06/12 07:00 | 118 | none

No hypoglycemia this admission.
Pt checks own glucose with glucometer independently (teach-back 06/10, RN T. Lam).

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
06/11 08:00 | 98.4 | 82 | 128/78 | 16 | 98% | RA
06/11 12:00 | 98.6 | 80 | 126/78 | 16 | 97% | RA
06/11 20:00 | 98.3 | 76 | 122/74 | 16 | 98% | RA
06/12 00:00 | 98.0 | 74 | 120/72 | 14 | 98% | RA
06/12 07:30 | 98.2 | 78 | 124/76 | 16 | 98% | RA

-- FICTIONAL DEMO RECORD - not a real patient --

Laboratory results

LABORATORY RESULTS

Collected | Test | Result | Ref
06/08 14:40 | Glucose | 438 H | 70-99 mg/dL
06/08 14:40 | Anion gap | 11 | 7-15
06/08 14:40 | Beta-hydroxybutyrate | 0.3 | <0.6 mmol/L
06/08 14:40 | WBC | 8.2 | 4.0-11.0 K/uL
06/09 05:00 | Hemoglobin A1c | 9.6 H | <5.7 %
06/11 05:00 | WBC | 7.4 | 4.0-11.0 K/uL
06/11 05:00 | Creatinine | 0.9 | 0.7-1.3 mg/dL
06/11 05:00 | CRP | 0.6 | <1.0 mg/dL

MICROBIOLOGY

06/09 06:00 MRSA nares PCR: NEGATIVE
06/12 06:15 SARS-CoV-2 rapid antigen: NEGATIVE
No wound culture sent (ulcer not clinically infected).

IMAGING

06/08 CXR PA/Lat: No acute cardiopulmonary process
06/09 R foot X-ray 3 views: No osteomyelitis. Soft tissue defect plantar 1st MT head.

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 13: Laboratory results

labs | orientation normal | legibility good

LABORATORY RESULTS

Collected | Test | Result | Ref
06/08 14:40 | Glucose | 438 H | 70-99 mg/dL
06/08 14:40 | Anion gap | 11 | 7-15
06/08 14:40 | Beta-hydroxybutyrate | 0.3 | <0.6 mmol/L
06/08 14:40 | WBC | 8.2 | 4.0-11.0 K/uL
06/09 05:00 | Hemoglobin A1c | 9.6 H | <5.7 %
06/11 05:00 | WBC | 7.4 | 4.0-11.0 K/uL
06/11 05:00 | Creatinine | 0.9 | 0.7-1.3 mg/dL
06/11 05:00 | CRP | 0.6 | <1.0 mg/dL

MICROBIOLOGY

06/09 06:00 MRSA nares PCR: NEGATIVE
06/12 06:15 SARS-CoV-2 rapid antigen: NEGATIVE
No wound culture sent (ulcer not clinically infected).

IMAGING

06/08 CXR PA/Lat: No acute cardiopulmonary process
06/09 R foot X-ray 3 views: No osteomyelitis. Soft tissue defect plantar 1st MT head.

-- FICTIONAL DEMO RECORD - not a real patient --

Discharge medication list

DISCHARGE MEDICATION RECONCILIATION
Patient: Daniel Doe MRN: LG-0572214
Continue / Resume
- Metformin 1000 mg PO twice daily with meals
- Atorvastatin 20 mg PO daily

New Medications
- Sitagliptin 100 mg PO daily
- Acetaminophen 650 mg PO every 6 hours PRN pain

Wound Supplies
- Silver alginate, bordered foam dressings (10 count), NS, cast cover

DME
- R post-op shoe (issued); own single-point cane

Stopped
- Enoxaparin (inpatient only)

No insulin at discharge. No antibiotics.
Patient able to self-administer all medications: Yes
30-day supply dispensed from outpatient pharmacy: Yes
Reviewed with patient by: S. Iyer, PharmD 06/12/2026

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 14: Discharge medication list

med_list | orientation normal | legibility good

DISCHARGE MEDICATION RECONCILIATION

Patient: Daniel Doe MRN: LG-0572214

Continue / Resume

- Metformin 1000 mg PO twice daily with meals
- Atorvastatin 20 mg PO daily

New Medications

- Sitagliptin 100 mg PO daily
- Acetaminophen 650 mg PO every 6 hours PRN pain

Wound Supplies

- Silver alginate, bordered foam dressings (10 count), NS, cast cover

DME

- R post-op shoe (issued); own single-point cane

Stopped

- Enoxaparin (inpatient only)

No insulin at discharge. No antibiotics.
Patient able to self-administer all medications: Yes
30-day supply dispensed from outpatient pharmacy: Yes
Reviewed with patient by: S. Iyer, PharmD 06/12/2026

-- FICTIONAL DEMO RECORD - not a real patient --